

Care of the Child or Young Person in Recovery Knowledge and Skills Workbook

Introduction to the National Knowledge and Skills Workbook for Post-Anaesthetic Care of Children and Young People

Purpose of the Competency Framework

This competency workbook is designed to provide a structured framework for the care of infants, children, and young people (CYP) in the Post-Anaesthetic Care Unit (PACU) across the United Kingdom. It outlines the essential knowledge, skills, and competencies required by professionals working in this critical area of care. This resource aims to standardise practice, ensure safe recovery for CYP post-surgery, and promote high-quality care in line with national healthcare standards.

National Standards and Guidelines

The framework aligns with the Royal College of Anaesthetists (RCOA) (2023) guidelines, the Royal College of Nursing (RCN) Workforce Standards (2025), and NHS Education for Scotland (2024) Core competency framework for Anaesthetic Practitioners. These standards emphasise the need for appropriately trained staff in PACU and ensure that service delivery is tailored to meet both the demand of the service and the needs of the people it serves.

Target Audience

This workbook is intended for healthcare professionals involved in the care of CYP in PACU settings, including:

- Registered Children's Nurses (RN Child)
- Registered Adult Nurses (RN Adult)
- Operating Department Practitioners (ODP)

The framework is designed to support a diverse group of professionals, regardless of their specific registration, by providing clear competencies essential for safe, effective, and child-centred recovery care.

The RCN (2013) is clear in the expectation that Registered Children's Nurses should be caring for CYP throughout their journey, and that there should be Registered Children's Nurses present in PACU. We agree with this statement but recognise that there are challenges with recruiting Registered Children's Nurses to PACU, particularly hospitals which have small

numbers of surgery in children. We acknowledge, however, the challenges in recruiting Registered Children's Nurses to PACU, particularly in hospitals where the volume of paediatric surgical cases is limited.

BARNA have not developed this workbook to discourage organisations from striving to meet this standard. The purpose of this workbook is to provide a structure and guide for individuals and organisations on the level of knowledge and skills for all professionals caring for CYP in recovery.

Scope and Limitations

This competency workbook provides guidance on the core knowledge and skills needed to care for CYP in PACU. However, it is important to note that this framework does not cover all aspects of professional development or mandatory training required for PACU practice. Additionally, while the core competencies listed here apply universally, individual units may need to adapt the framework to their specific needs and the level of surgery performed (e.g., removing competencies such as tracheostomy care, or adding in specialised cardiac competences where applicable).

The Importance of Competency in the PACU Setting

PACU is a high-risk environment where patients, especially children, require specialised care post-surgery. Competency in this setting is critical for ensuring the safe and effective recovery of CYP. Skilled professionals are essential for identifying early signs of complications, providing timely interventions, and optimising recovery outcomes. The competencies outlined in this workbook are foundational for maintaining patient safety, improving care quality, and ensuring positive recovery experiences for CYP.

National and Local Context

This workbook is intended for use in PACU settings across the United Kingdom. Recognising that service delivery can vary between units, whether in large regional hospitals or smaller settings, this framework is adaptable to the specific needs and challenges of different institutions. While the core competencies are standardised, flexibility is provided for units to add or remove competencies based on their specific patient needs and the complexity of surgeries they provide.

Collaboration and Stakeholder Engagement

The framework for this competency workbook has been used with thanks to the East of England, Operational Delivery Network for Children in Surgery, who had developed it for use with their Paediatric Post Anaesthetic Care Unit (PACU) course.

The development of this workbook has been a collaborative effort involving experts from The Association of Anaesthetists of Great Britain and Ireland, The Association for Perioperative Practice, The Centre for Perioperative Care and The Royal College of Nursing. This ensures that the competencies outlined reflect best practices and meet the needs of both practitioners and patients.

No conflict of interests were declared.

Endorsement

This document has been endorsed by:

- **The Association For Perioperative Practice**
- **The Royal College of Anaesthetists**
- **The Royal College of Nursing**

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Daniel Gooding, Trainee Paediatric Advanced Clinical Practitioner, Torbay and South Devon NHS Foundation Trust. Royal College of Nursing, Acute Care Forum Chair.

Lindsay Keeley, Clinical, Patient Safety & Quality Lead, The Association for Perioperative Practice.

Sophie McKay, Lecturer in Clinical Skills and Simulation, Nursing & Midwifery Department, Middlesex University. Royal College of Nursing, Acute Care Forum member.

Alison Popa, PeriAnaesthesia Nurse Educator. BARNA Paediatric Lead.

Commitment to Continuous Improvement

This workbook is part of an ongoing commitment to improving the quality of care for CYP in PACU. As clinical practices, evidence, and national guidelines evolve, the framework will be reviewed and updated regularly to maintain its relevance and ensure that it reflects the latest advancements in paediatric anaesthesia and recovery care.

Assessment

The supervisor should be a registered professional and have 2 or more years of recent experience working in PACU or be a Consultant Paediatric Anaesthetist. We recognise that not all PACU units employ registered children's nurses, and as such it may be necessary to have more than one supervisor. We recommend that child specific skills (marked with "RNC") be signed off by a registered children's nurse, who has completed High Dependency Care Competency. It is not essential that this person is working within the PACU environment.

This Knowledge and Skills Workbook does not include all the knowledge and skills required for the role, nor mandatory training. However, it should be used as part of continuing personal and professional development, for registered practitioners who care for CYP in the PACU.

Feedback and Engagement

We encourage feedback from all users of this workbook. Contributions and feedback based on experience will help enhance the relevance and utility of the framework. Please direct any comments or suggestions to connectwithbarna@googlemail.com. This document will be reviewed every two years (next review date 1/10/2027)

References

Benner, P. (1984). From novice to expert: Excellence and power in clinical nursing practice. Upper Saddle River, N.J.: Prentice Hall.

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Royal College of Nursing (2025) Nursing workforce standards. Available at: <https://www.rcn.org.uk/Professional-Development/publications/rcn-nursing-workforce-standards-uk-pub-011-930> (Accessed: 04.09.2025).

Benner's Stages of Clinical Competence

Guidance for Supervisors

BARNA recommends using Benner's stages of clinical competence (Benner, 1984) when assessing competence. Supervisors should evaluate candidates against the criteria below and indicate their level of practice in the box titled 'level' using the abbreviations provided.

It is expected that nurses and Operating Department Practitioners (ODPs) will be deemed competent in all components of this document to pass the competency assessment.

Competency Levels

- **Novice (N)**
Has little or no experience in the situations they are expected to perform in. Lacks confidence to demonstrate safe practice and requires constant verbal and physical cues to complete tasks.
- **Advanced Beginner (AB)**
Able to draw on previous experiences to inform decision-making and practice. Performance is marginally acceptable. Efficient and skilful in some aspects, but requires supportive cues. Knowledge is developing.
- **Competent (C)**
Demonstrates efficiency, coordination, and confidence. Care is completed in a timely fashion without the need for supportive cues.
- **Proficient (P)**
Perceives situations as a whole rather than in parts. Applies prior experiences to different situations and can anticipate necessary changes.
- **Expert (E)**
Has intuitive understanding and critical appraisal of situations. Operates with deep understanding and performs accurately, flexibly, and with high proficiency.

Note

Throughout this document, teaching words and derivatives such as 'CYP' (Children and Young Person), 'paediatric', 'infant', 'child', and 'young person' are used interchangeably to refer to care provided in a hospital setting.

Benner, P. E. (1984) From Novice to expert: Excellence and power in clinical nursing practice. Addison-Wesley Pub Co: California

Date:	Initial meeting
Date:	Mid-point meeting
Date:	Final meeting

Assessor signatory Log

Name	Role	Signature	Initials

1.0 Essential Skills	Supervisor Sign	Date
1.1 Successful completion of Paediatric Immediate Life Support Course (pILS) or equivalent (please state) within the last 12 months <i>Date of course:</i> _____		
1.2 Demonstrates awareness and knowledge of the paediatric resus trolleys, its contents and location. _____		
1.3 Has completed national PEWS training, available on E-learning for Health _____		
1.4 Up to date with all local mandatory training _____		
1.5 Has completed Safeguarding for children level 3 training _____		
1.6 Completed preceptorship programme and associated competencies. (If N/A please give reason) _____		

2.0 Essential Patient Care	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
2.1 Is able to identify admission protocol for PACU, and identify patients not suitable for admission to PACU				
2.2 Is able to demonstrate the importance of creating a child friendly environment within PACU				
2.3 Is able to apply age-appropriate communication across the lifespan. Is able to educate and inform patients and carers attending PACU (RNC)				
2.4 Is able to discuss and demonstrate adaptations which may be required for people with neurodiversity (RNC)				
2.5 Is able to discuss and demonstrate adaptations which may be required for people with learning disabilities (RNC)				
2.6 Ensures that the principles of Family Centred care are maintained within the PACU environment. Is able to involve the patient and carers are actively involved in care (RNC)				

2.7 Is able to prepare a PACU bedspace for a patient, considers all the requirements for, as a minimum: • Patient safety • Equipment • Documentation • Placement of patient bed on arrival	
2.8 Consistently correctly documents appropriately for a child in PACU: • Observations • Validated early warning score • Pain Assessment • Wound Assessment • Pressure area • Discharge criteria	
2.9 Is able to demonstrate rationale for frequency of observations including Paediatric Early Warning System (PEWS) score and discuss the actions to be taken depending on this	
2.10 Demonstrates sound knowledge and competence in the use of essential equipment and monitoring used within PACU	
2.11 Appropriately escalates when concerned about a child in line with organisational escalation pathways and national PEWS (RNC)	

2.12 Is able to explain the significance of discharge criterion for patient wellbeing. Is able to identify patients fit for discharge, and that the discharge criteria is met in a way which is person specific	
2.13 Demonstrates an understanding of the principles of advocacy (RNC)	
2.14 Demonstrates knowledge and understanding of consent issues with CYP (RNC)	
2.15 Is able to discuss the principles of premedication including: 1. Rationale 2. Contraindications 3. Implications in post anaesthetic phase	
2.16 Is able to discuss how and why modes of anaesthesia are used, and the implications of this for the care of CYP in PACU: 1. IV 2. Inhalational 3. Rapid sequence induction 4. Total Intravenous Anaesthesia	

3.0 Assessment & management of Airway	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
3.1 Is able to apply the knowledge of the anatomy of the upper airway of the infant, child and young person to management of the airway in PACU, including physiological changes to the airway postoperatively as a consequence of anaesthesia and surgery. For example, the importance of the large occiput in infants, and the use of a shoulder roll and head ring.				
3.2 Is able to describe the signs of complete and partial airway obstruction				
3.3 Is able to demonstrate the ability to maintain a patent airway using head tilt, chin lift for: • Infant (neutral position) • Child • Adolescent Is able to demonstrate an awareness of hand and finger position and the potential complication of airway compression				

3.4 Is able to demonstrate the ability to maintain a patent airway using jaw thrust for: • Infant (neutral position) • Child • Adolescent	
3.5 Is able to demonstrate the ability to optimise a patent airway using positioning • Infant (neutral position) • Child • Adolescent	
3.6 Identify abnormal respiratory noises and suggest causation and care implications for each: • Stridor • Snore • Stertor • Wheeze • Grunt	
3.7 Demonstrates the ability to perform oral & naso-pharyngeal suction to clear secretions. Is able to demonstrate ability to determine suction catheter sizing and discuss the suction pressures used and demonstrate safe practice	

3.8 Demonstrate how to prepare an adrenaline nebuliser	
3.9 Describe effective positioning for child receiving nebulised adrenaline	
3.10 Discuss anticipated changes in condition associated with nebulised adrenaline, comment on anticipated changes for child both improving and deteriorating	
3.11 Discuss the need for minimal handling to limit distress, and how this can be achieved in the PACU environment	
3.12 Discuss the implications of reduced level of consciousness on the patients airway	

4. Airway Adjuncts	Evidence (use additional sheet as required)	Supervisor Yes / No (level not required)	Date
Demonstrate how to size and insert an oropharyngeal airway: • Infant • Child • Adolescent			
Demonstrate the safe care and removal of a patient with an oropharyngeal airway: • Infant • Child • Adolescent			
4.3 Oropharyngeal airway: Is able to discuss the rationale for using an oropharyngeal airway, including benefits, contraindications and potential complications			

4.4 Nasopharyngeal airway:

Demonstrate how to size and insert an nasopharyngeal airway:

- Infant
- Child
- Adolescent

4.5 Nasopharyngeal airway:

Demonstrate the safe care, suctioning and removal of a patient with an nasopharyngeal airway:

- Infant
- Child
- Adolescent

4.6 Nasopharyngeal airway:

Is able to discuss the rationale for using an nasopharyngeal airway, including benefits, contraindications and potential complications

4.7 Laryngeal mask airway / igel:

Demonstrate how to size and insert a Laryngeal mask airway / igel:

- Infant
- Child
- Adolescent

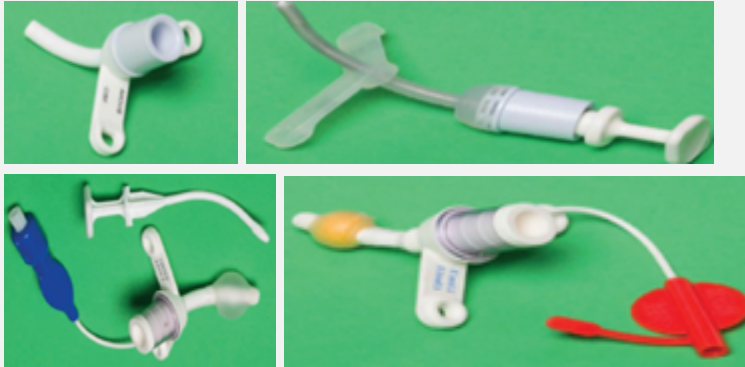
4.8 Laryngeal mask airway / igel:

Demonstrate the safe care, suctioning and removal of a patient with an oropharyngeal airway:

- Infant
 - Child
 - Adolescent
-

4.10 Tracheostomy:

Identify the following tubes and their features:



4.11 Give rationale for spare tubes and equipment carried in emergency box

4.12 Describe what symptoms a patient with a blocked or displaced tube may display and explain the management for both

4.13 Is able to articulate the importance of suctioning as a first manoeuvre when assessing tracheostomy patency

4.14 Discuss the resuscitation algorithm for a child with a tracheostomy

4.15 Carry out a tracheostomy tube change

5.0 Assessment and management of Breathing	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
5.1 Can consistently and comprehensively assess patient's breathing, including: ● Respiratory rate ● Respiratory rhythm ● Depth of breathing ● Chest movement ● Effort of breathing and use of accessory muscles (tracheal tug, intercostal recession, subcostal recession, sternal recession) ● Efficacy of breathing ● Skin colour / perfusion ● Chest auscultation ● Additional noises ● (RNC)				
5.2 Demonstrates knowledge of the normal respiratory parameters for children of different ages in line with national PEWs and the resuscitation council (RNC)				
5.3 Appropriately measure and interpret oxygen saturation monitoring. Comment on:				

5.4 Is able to discuss oxygen requirements in the immediate postoperative period	
5.5 Discuss underlying causes of apnoea & management options	
5.6 Is able to discuss respiratory complications in infants post anaesthetic and list the possible causes of postoperative cyanosis	
5.7 Carbon Dioxide monitoring: • Is able to discuss appropriate use of expired Carbon Dioxide monitoring • Is able to discuss CO2 waveform and explain causes for differing waveforms	

5.8 Is able to discuss the indications, benefits and uses for the use of different oxygen delivery devices: Understands the rationale, and can demonstrate administration of oxygen at the correct % via: • Simple face mask • Nasal Cannula • Blow by oxygen • Face Mask with reservoir bag • Mapleson F and Mapleson C anaesthetic circuit	
5.9 Is able to demonstrate appropriate use of a Bag valve mask, including provide rationale for size of mask and size of bag	
5.10 Is able to hold a face mask in position, ensuring a gas-tight seal, or perform manual intermittent positive pressure ventilation	
5.11 Is able to demonstrate bag-mask ventilation for ventilatory support both: • BVM • Anaesthetic circuit Including the importance of the use of a two person technique	

5.12 Is able to demonstrate anaesthetic circuit ventilation, including the pressure valve / cut bag for: • Mapleson F • Mapleson C (or equivalent for your organisation)	
5.13 Identifies signs of respiratory distress	
5.14 Identifies signs of hypoxia	
5.14 Demonstrates the ability to deal with adverse respiratory reactions / events correctly	
5.15 Is able to set up intubation equipment and explain the differences in equipment required for safe intubation of children and infants	

6.0 Arterial, venous and capillary blood gases	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
6.1 State the normal values for arterial or venous blood gas samples				
6.2 Can undertake basic interpretation of an arterial or venous blood gas in the context of the patient condition, commenting as a minimum on pH, PaCO ₂ , PaO ₂ , Bicarbonate, lactate and able to recognise respiratory acidosis, metabolic acidosis, and other values produced such as glucose and electrolytes				
6.3 Can escalate effectively and transcribe results into patient notes				

6.4 Interpret the following blood gases via an ABCDE approach, stating whether metabolic or respiratory, acidosis or alkalosis. Briefly discuss the anticipated management.

A. 2-year-old child post PICC line insertion following diagnosis of pneumonia and requiring IV antibiotics. They have increased work of breathing with subcostal and intercostal recession, oxygen saturations on air are 91%.

- pH 7.25
- PCO_2 8.2
- PO_2 6.8
- HCO_3 27
- Base 2
- Lactate 1.8

B. 6yr old girl, history of facial abscess 2 weeks ago, did not complete course of antibiotics as felt better and didn't like the taste, now presenting with fever, tachycardia and a blanching rash. Has come to theatre today for a tooth extraction and abscess drainage.

- pH 7.16
- PCO_2 4.9
- PO_2 6.5
- HCO_3 16
- Base 8
- Lactate 3.2

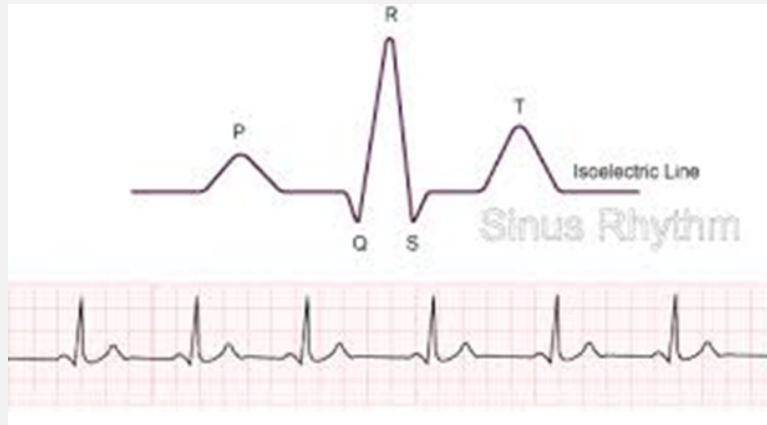
C. 3 week old infant post pyloromyotomy, when reviewing the notes the last blood gas results are noted to be:

- pH 7.51
- PCO_2 5.01
- PO_2 8.2
- HCO_3 33
- Base 3.6
- Lactate 4

7.0 Cardiovascular assessment and management	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
7.1 Demonstrates the ability to perform an accurate assessment of central and peripheral pulse including: • Rate • Rhythm • Volume (RNC)				
7.2 Consistently and comprehensively assesses cardiovascular status, including: • Skin temperature and peripheral perfusion • Central and peripheral capillary refill time • Colour (RNC)				

7.3 Blood pressure: Is able to select correct cuff and perform non-invasive blood pressure monitoring, giving rationale for selection of limb and frequency of monitoring Can carry out a manual blood pressure measurement (RNC)	
7.4 Demonstrates knowledge of normal parameters and symptoms of cardiovascular compromise (RNC)	
7.5 Discuss the aspects to anatomical and physiological changes to the cardiovascular system postoperatively as a consequence of anaesthesia and surgery in CYP (RNC)	
7.6 Is able to discuss the common circulatory problems encountered in the immediate postoperative period	
7.7 Demonstrate correct placement of leads for cardiac monitoring	

7.8 Label the components of this rhythm and identify it.



7.10 Draw the following rhythms and state the characteristics and appropriate course of action for each.

- VF
- VT
- PEA
- Asystole
- SVT

7.11 Demonstrates the ability to measure arterial pressure and recognise deviations from the norm

7.12 Understands the importance of mean arterial pressure in CYP (RNC)

7.13 Understands the use of vasoactive drugs to treat heart rate and arterial pressure in the PACU	
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8.0 Fluid management	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
8.1 Can accurately calculate the fluid requirement for a child receiving IV fluids (RNC)				
8.2 Demonstrate the ability to interpret fluid balance status (RNC)				
8.3 Discuss the rationale for maintaining strict input and output records and calculating accurate fluid balance (RNC)				
8.4 Explain how you would identify the signs of dehydration and discuss why young children are at greater risk of dehydration (RNC)				
8.5 Can accurately calculate urine output in ml/Kg/Hr and discuss normal parameters (RNC)				
8.6 Demonstrates the ability to identify factors stimulating PONV				

8.7 Discuss treatment of PONV using pharmacological and non-pharmacological methods to provide care and support for the patient	
8.8 Demonstrates appropriate knowledge and application of protocols for IDDM patients undergoing surgery (including blood glucose monitoring and fluid management and recognition of hypoglycaemia in infants and children)	
8.9 Discuss the rationale for, and how in practice to give replacement/ deficit fluids in addition to maintenance fluids	
8.10 Demonstrate correct procedure for placement of a nasogastric tube, including explaining the differences in calculating insertion depth for under 1 year and over 1 years of age (RNC)	
8.11 Demonstrate correct procedure & frequency for checking placement of a nasogastric tube (according to local guidelines)	
8.12 Demonstrates safe care of a child with a gastrostomy	

9.0 Neurological assessment and management	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
9.1 Identify potential neurological complications following surgical intervention				
9.2 Discuss the management of impaired neurological status in the immediate post anaesthetic phase				
9.3 Describe the appropriate use of neurological assessment tool pre- and post-operatively i.e. Glasgow coma score or ACVPU				
9.3 Describe and demonstrate the elements of full neurological observations, give rationale for each				
9.4 Can reliably complete a full set of neurological observations and chart appropriately, and is able to discuss the use of an adapted GCS chart for infant and paediatric patients				

9.5 Can describe different types of seizure activity and articulate the action required in each case. Can demonstrate knowledge of the seizure management algorithm	
9.6 Discuss the side effects and considerations for administration of: • Buccal midazolam • PR anti convulsant • IV anti convulsant	
9.7 Identify the signs and symptoms associated with raised intracranial pressure including when it may be suspected, including provision of CYP neuroprotective measures according to local guidance	
9.8 Identify the stages of emergence from anaesthesia and discuss how to care for the patient through these stages	

10 Environmental / exposure assessment and management.	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
10.1 Discuss the importance of performing a complete top to toe assessment with a parent / guardian / chaperone present				
10.2 Discuss the normal range for core temperature, and the potential effects of this in a post-operative patient				
10.3 Is able to discuss the action to be taken if outside of the expected norms				
10.4 Discuss situations that would raise safeguarding concerns in the perioperative setting (RNC)				

11.0 Pain Management	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
11.1 Can describe pain pathways including physiology of pain transmission				
11.2 Is able to describe ways to assess severity of acute pain, including discussing the benefits and uses of different pain assessment tools (for example: FLACC, modified FLACC, Wong and Baker faces and the number scale)				
11.3 Demonstrates the ability to perform an accurate assessment of pain using FLACC, modified FLACC, Wong and Baker faces and the number scale, including utilisation of the parent and carers viewpoint				
11.4 Discuss non-pharmacological methods of pain management (distraction, guided imagery, parental support, play)				
11.5 Describes the analgesic ladder and identifies appropriate post-operative analgesic regimes including types of drugs and doses				

11.6 Explains how to manage rescue analgesia for the patient with severe pain	
11.7 Lists the complications of analgesic drugs	
11.8 Demonstrates the ability to prepare and commence • PCA/ NCA • Epidural • Local anaesthetic infusion	
11.9 Demonstrates the ability to care for a child receiving • PCA/ NCA • Epidural • Local anaesthetic infusion	
11.10 Demonstrates ability to care for a patient with neuraxial anaesthesia or peripheral nerve block	
11.11 Demonstrates an understanding of and ability to manage side effects and complications of analgesics.	

12.0 Medication management	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
12.1 Can calculate appropriate doses and concentrations of drugs for paediatrics.				
☐ Read local paediatric medication administration policy				
12.2 Is able to demonstrate the safe administration of medication to infants, children and young people, including the importance of consent and negotiation				
12.3 Is able to articulate the indications, contraindications and side effects of medication used in anaesthesia:				
• Propofol • Volatile gases • Muscle relaxants • Midazolam • Opiates				

13.0 Complications of Anaesthesia	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
Is able to discuss the indications, implications and treatment for the following anaesthetic complications:				
13.1 Laryngospasm				
13.2 Incomplete reversal / neuromuscular blockade				
13.3 Malignant hyperthermia				
13.4 Local anaesthetic systemic toxicity				
13.5 Pulmonary oedema				

13.6 Bronchospasm	
13.7 Anaphylaxis	
13.8 Vasovagal response	
13.9 Pulmonary Aspiration	
13.10 Hypovolaemia	
13.11 Emergence delirium	
13.12 Delayed Emergence	

13.13 Opiate Toxicity	
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14.0 Complications of Surgery	Evidence (use additional sheet as required)	Supervisor		Date
		Benner Level	Sign	
Is able to discuss the indications, implications and treatment for the following anaesthetic complications:				
14.1 Major haemorrhage				
14.2 Surgical emphysema				
14.3 Pneumothorax				
14.4 Bleeding post Adenoidectomy and/or Tonsillectomy				
14.5 Compartment syndrome				

14.6 Inadvertent Perioperative hypothermia	
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PACU Care Knowledge & Skills Workbook Continuation sheet (print number required & attach to original)

Element Indicator (eg. 10.7)	Evidence	Supervisor		Date
		Benner Level	Sign	

Element Indicator (eg. 10.7)	Evidence	Supervisor		Date
		Benner Level	Sign	

Continuing Skills Evaluation

All Registered practitioners have a duty of care to their patients and as such are periodically required to undergo monitoring and appraisal. It is suggested that each practitioner perform an evaluation of their practice 6 months after completing this workbook and as part of their annual appraisal and review.

Date	Six-months Post Completion of this Workbook
I believe that I have maintained my knowledge and skills required to care for CYP in PACU ☐ Strongly agree ☐ Agree ☐ Neither agree nor disagree ☐ Disagree ☐ Strongly disagree Comments / areas for further development Signature of supervisor:	

Date

Annual appraisal review

I believe that I have maintained my knowledge and skills required to care for CYP in PACU

☐ Strongly agree

☐ Agree

☐ Neither agree nor disagree

☐ Disagree

☐ Strongly disagree

Comments / areas for further development

Signature of supervisor:

Glossary of acronyms

- **ABCDE** approach Airway, breathing, circulation, disability and exposure structure to assessment
- **ACVPU** Awake, Confused, Voice, Pain, Unconscious
- **BVM** Bag valve mask
- **CYP** Children and young people
- **FLACC** Faces, legs, activity, cry, consolability scoring system
- **GCS** Glasgow coma scale
- **HCO₃** Bicarbonate
- **IDDM** Insulin dependant diabetes mellitus
- **IV** Intravenous
- **ml/Kg/Hr** Millilitres per kilogram per hour
- **PO₂** Partial pressure of Oxygen
- **PCA/NCA** Patient controlled analgesia system / Nurse controlled analgesia system
- **PCO₂** Partial pressure of Carbon Dioxide
- **PEA** Pulseless electrical activity
- **pH** Power of Hydrogen
- **PICC** Peripherally inserted central catheter
- **PONV** Post operative nausea and vomiting
- **PR** Per Rectum
- **SVT** Supraventricular tachycardia
- **VF** Ventricular fibrillation
- **VT** Ventricular tachycardia

BARNA quality assurance

This is a BARNA document, which has been endorsed by the Association for Perioperative Practice, Royal College of Anaesthetists and Royal College of Surgeons.

Impact assessment

For a Paediatric PACU competency framework, impact could be seen in:

- Nurse confidence and self-reported competence
- Observable clinical practice improvements
- Patient safety indicators (e.g. reduction in airway complications, escalation rates)
- Education and training efficiency (e.g. reduced time competency sign off)

Evaluation methods

- Survey / questionnaires
- Pre and post implementation surveys of nurses on confidence, perceived knowledge and the usability of the framework

Impact assessment

For a Paediatric PACU competency framework, impact could be seen in:

- This competency framework will be evaluated through periodic surveys, audits and feedback from PACU nurses and educators.
- Outcomes to be measured include nurse confidence, progression against competencies and impact on clinical practice.
- Findings will inform future revisions and updates to the framework.

